

RIVERDALE COMMUNITY HOSPITAL

Department of Pulmonology & Critical Care

DISCHARGE SUMMARY

Document No: DS-2024-00998	Admission Date: March 12, 2024	Discharge Date: March 17, 2024
Attending Physician: Dr. Michael Torres, MD, FCCP		

Patient Name: Harold Dean Kowalski	Date of Birth: April 5, 1946 (Age: 77)
MRN: RCH-221-5509	Sex: Male
Insurance: Medicare Part A & B + Medigap	Discharge Disposition: Senior Living — Riverbend Assisted Living

PRIMARY DIAGNOSIS

Acute exacerbation of COPD (AECOPD), severe — GOLD Stage III

Secondary: Hypertension, Coronary artery disease (s/p CABG 2018), BPH, Osteoporosis

REASON FOR ADMISSION

Patient presented on 3/12 with worsening dyspnea, increased sputum production (yellow-green), and decreased exercise tolerance over 4 days. O2 saturation on arrival: 84% on room air. ABG: pH 7.32, PaCO2 58, PaO2 54. Admitted for acute exacerbation management and initiation of supplemental oxygen.

HOSPITAL COURSE

Patient treated with systemic corticosteroids (methylprednisolone IV x3 days then prednisone PO taper), azithromycin for presumed bacterial exacerbation, and aggressive bronchodilator therapy. Patient required BiPAP support for 18 hours during admission. Oxygen supplementation required throughout. At discharge, patient's O2 saturation stable at 92% on 2L/min nasal cannula. Pulmonary function testing deferred — to be arranged outpatient.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
Tiotropium (Spiriva) (NEW)	18mcg	Inhaled	Once daily	HandiHaler device — educate on technique
Fluticasone/Salmeterol (NEW)	250/50mcg	Inhaled	Twice daily	Rinse mouth after use — thrush prevention
Albuterol MDI (Rescue)	2 puffs	Inhaled	PRN	FREQUENCY NOT SPECIFIED — see flag below
Prednisone (TAPER)	40mg	PO	See schedule	40mg x2d, 30mg x2d, 20mg x2d, 10mg x2d, STOP
Azithromycin	250mg	PO	Once daily x3 more days	Complete course — ends 3/20
Amlodipine	10mg	PO	Once daily	Continue unchanged
Lisinopril	5mg	PO	Once daily	Continue — hold if SBP <90
Tamsulosin	0.4mg	PO	Once daily at bedtime	For BPH
Alendronate	70mg	PO	Once weekly (Monday)	Take with full glass of water, remain upright 30min
Omeprazole (NEW)	20mg	PO	Once daily	GI protection during steroid taper

OXYGEN REQUIREMENTS

Supplemental oxygen: 2L/min via nasal cannula at rest.

Titrate to maintain O2 saturation 88-92% (not higher — hypercapnic patient).

Oxygen during activity: Patient may require higher flow — assess with ambulation.

Home oxygen order transmitted to DME supplier: AirLife Medical, 301-555-3300.

[!] Oxygen flow rate during sleep NOT specified.

[!] DME order confirmation NOT provided — facility to verify delivery with AirLife Medical.

[!] Albuterol PRN: frequency and maximum daily doses NOT documented — contact prescriber.

EQUIPMENT NEEDS

Nebulizer machine — for albuterol if patient unable to use MDI effectively.

Portable oxygen concentrator — for ambulation and transport.

Pulse oximeter — for continuous saturation monitoring.

[!] Nebulizer order NOT placed — facility to confirm if MDI technique adequate.

[!] Portable oxygen concentrator NOT ordered — facility to clarify with DME supplier.

FOLLOW-UP

1. Pulmonology (Dr. Torres): 2 weeks — 301-555-4400

2. PCP (Dr. Nguyen): 1 week — 301-555-1100

3. Pulmonary function testing: arrange outpatient with pulmonology

[!] Neither appointment scheduled at time of discharge.

ALLERGIES

Penicillin — hives

Aspirin — bronchospasm (NSAID-exacerbated respiratory disease)